

Question:

A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?

Answer Choices:

- A. The patient is at risk for double depression.
- B. The patient may have symptoms of mania or psychosis.
- C. The patient is likely to show anhedonia.
- D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

<think>

Finding Reasoning Paths :

1 . **Persistent sad mood , hope lessness , suicidal ide ation , and guilt** â€” Major Dep ressive Disorder (MDD).

2 . **Difficulty making decisions , impaired motivation , low energy , difficulty concentrating , and suicidal ide ation** â€” Major Dep ressive Disorder .

3 . **Re current suicidal ide ation , sadness , and hope lessness for over 2 weeks** â€” Major Dep ressive Disorder .

4 . **Not responding to job demands , diminished motivation , and suicidal ide ation** â€” Neuro logical or psychiatric condition such as major depressive disorder or psychosis .

5 . **Persistent symptoms for over 2 weeks with no mention of other positive symptoms** â€” Rule out man ia , psychosis , or substance use -induced psychosis .

Reasoning Process :

1 . The patient reports **persistent sadness for a long time** , feeling cursed , and experiencing **su ic idal ide ation** , which are hallmark features of **major depressive disorder (MDD)** . These symptoms are consistent with a depressive mood disorder .

2 . Additional symptoms include **hope lessness** , **gu ilt** , **po or concentration** , **limited motivation** , **energy decrease** , and **difficulty making decisions** , which further support the diagnosis of **MDD** .

3 . The patient has **su ic idal ide ation** and **rec urrent feelings of hope lessness** , which are significant red flags for severe depression . These symptoms are not typical of other conditions such as Parkinson 's disease or chronic cere brosp inal ven ous ins ufficiency , as these conditions do not usually present with persistent sadness , hope lessness , or guilt .

4 . The patient 's symptoms have been ongoing for over **2 weeks** , which is the minimum duration required for a diagnosis of major depressive disorder in the DSM - 5 .

5 . The patient is currently **function ing with a job** and getting enough sleep , but his functional status is likely impaired by his depressive symptoms . There is no mention of physical symptoms such as rig idity , brady kines ia , or other neurological findings that would suggest an underlying neurological or vascular condition like Parkinson 's disease .

6 . The patient 's symptoms do not suggest **psychosis** , as there is no mention of del usions , halluc inations , or disorganized thinking . Similarly , there is no indication of a substance -induced state or a manic episode , as these would typically present with at least one month of alternating mood episodes .

7 . The patient 's presentation align s most closely with **major depressive disorder** rather than another psychiatric or neurological condition .

Conclusion :

The patient is most likely experiencing **major depressive disorder** . This conclusion is supported by the persistence of symptoms (over 2 weeks) , presence of hallmark depressive symptoms (sad ness , hope lessness , suicidal ide ation , guilt) , and the absence of other findings that would suggest alternative diagnoses .

</think>

< answer >

The patient is at risk for double depression .

</answer>